



Australian Government

**Commonwealth
Standard Grant Agreement**

between
the Commonwealth represented by
Department of Health and Aged Care
and
Urapuntja Health Service Aboriginal
Corporation

Grant Agreement

Once completed, this document, together with each set of Grant Details and the Commonwealth Standard Grant Conditions (Schedule 1), forms an Agreement between the Commonwealth of Australia (the Commonwealth) and the Grantee.

Parties to this Agreement

The Grantee

Full legal name of Grantee	Urapuntja Health Service Aboriginal Corporation
Legal entity type (e.g. individual, incorporated association, company, partnership etc)	Indigenous Corporation
Trading or business name	Urapuntja Health Service Aboriginal Corporation
Any relevant licence, registration or provider number	
Australian Company Number (ACN) or other entity identifiers	
Australian Business Number (ABN)	45 449 518 275
Registered for Goods and Services Tax (GST)	Y
Date from which GST registration was effective	
Registered office (physical/postal)	Sandover Highway, UTOPIA NT 0872
Relevant business place (if different)	
Telephone	08 8956 9875
Fax	08 8956 9863
Email	ceo@urapuntjahealthservice.com

The Commonwealth

The Commonwealth of Australia represented by Department of Health and Aged Care
23 Furzer Street PHILLIP ACT 2606
ABN 83 605 426 759

Background

The Commonwealth has agreed to enter into this Agreement under which the Commonwealth will provide the Grantee with one or more Grants for the purpose of assisting the Grantee to undertake the associated Activity.

The Grantee agrees to use each Grant and undertake each Activity in accordance with this Agreement and the relevant Grant Details.

Scope of this Agreement

This Agreement comprises:

- (a) this document;
- (b) the Supplementary Terms from the Clause Bank (if any);
- (c) the Standard Grant Conditions (Schedule 1);
- (d) the Grant Details;
- (e) any other document referenced or incorporated in the Grant Details.

Each set of Grant Details, including Supplementary Terms (if any), only applies to the particular Grant and Activity covered by that set of Grant Details and a reference to the 'Agreement' in the Grant Details or the Supplementary Terms is a reference to the Agreement in relation to that particular Grant and Activity. If there is any ambiguity or inconsistency between the documents comprising this Agreement in relation to a Grant, the document appearing higher in the list will have precedence to the extent of the ambiguity or inconsistency.

This Agreement represents the Parties' entire agreement in relation to each Grant provided under it and the relevant Activity and supersedes all prior representations, communications, agreements, statements and understandings, whether oral or in writing.

Certain information contained in or provided under this Agreement may be used for public reporting purposes.

Grant Details

Organisation ID:	2-M9NY76
Agreement ID:	4-JOVK9IV
Program Schedule ID:	4-JOVK9IX

A. Purpose of the Grant

The purpose of the Grant is to:

The purpose of this Grant is to fund Aboriginal Community Controlled Health Organisations (ACCHOs) that are currently delivering Comprehensive Primary Health Care (CPHC) under the Indigenous Australians' Health Programme (IAHP) to continue to deliver culturally appropriate CPHC tailored to health needs and priorities of First Nations peoples.

This Grant was developed in response to a government commitment to introduce four-year rolling funding agreements from 1 July 2024 for ACCHOs funded under the IAHP to deliver CPHC.

This Grant is being provided under, and these Grant Details form part of, the Agreement between the Commonwealth and the Grantee.

The Grant is being provided as part of the Indigenous Australians Health Program program

Activity Title: Comprehensive Primary Health Care

Activity ID: 4-JOW2645

B. Activity

The activity aims to:

Objective 1: provide First Nations people with access to culturally appropriate comprehensive primary health care services. Outcome: First Nations people can access culturally appropriate comprehensive primary health care services.

Objective 2: support ACCHOs to deliver culturally appropriate comprehensive primary health care services tailored to the health needs and priorities of their communities. Outcome: ACCHOs deliver culturally appropriate, comprehensive primary health care services that meet the health needs and priorities of their local communities.

Objective 3: support ACCHOs to embed Continuous Quality Improvement (CQI) and best practice clinical and organisational governance into CPHC service delivery, supporting good health outcomes for clients and service sustainability. Outcome: ACCHOs deliver best practice comprehensive primary health care services to their local communities.

The Grantee must provide essential primary health care, including general practice and clinical services delivered by suitably qualified medical professionals.

The Grantee has the flexibility to determine what other eligible services, as outlined in the Grant Opportunity Guidelines, will be delivered in order to meet identified community health and wellbeing needs, and determine the appropriate workforce required to implement those services. Those services may include:

- population health programs;
- activities that support the delivery of essential clinical services;
- improving access to antenatal care and child, maternal and family health services by First Nations children, their mothers and families;
- the prevention, detection and management of chronic diseases;
- investment in priority health areas in regions of high health need or population growth; and
- improving the clinical effectiveness of the health system and supporting sustainable, long term service reform delivery and improvement through Continuous Quality Improvement (CQI).

The Grantee must comply with any business improvement plan or remediation plan requirements, including additional reporting, if requested by the Department.

Grant Period

The grant period consists of an initial grant term of four years from 2024-25 to 2027-28, with funding amounts stipulated for 2024-25 and 2025-26.

Every two years the Grantee's CPHC grant funding levels and performance against this grant will be reviewed. The IAHP Primary Health Care Funding Distribution Model will be applied to inform the review of Commonwealth Standard Grant Agreement 2022

each ACCHO's CPHC funding levels under this grant.

The first review will take place in 2025-26. Should the Grantee meet minimum eligibility requirements, the department may at its discretion offer a grant extension by a Deed of Variation, extending the funding term until 2029-30.

A further two-year extension may be considered in 2027-28, extending the grant end date to 30 June 2032.

Minimum Eligibility Requirements for Grant Extension

To be considered for an extension under the rolling funding agreements, grantees must meet minimum eligibility requirements:

1. Maintain compliance with Eligibility requirements and maintenance of relevant skills, qualifications and checks as set out at Section 4 of GO6742 - IAHP Comprehensive Primary Health Care - Rolling Funding Arrangements Grant Opportunity Guidelines
2. Timely and satisfactory submission of all reporting deliverables set out in this Agreement, including:
 - Activity Work Plan (AWP) (initial submission with annual updates);
 - Annual performance report against AWP (annually);
 - OSR (annually);
 - nKPI (bi-annually);
 - Financial Acquittal Report (annually); and
 - Compliance with all terms and conditions of this Agreement.

Funding Calculations

Funding for 2024-25 will equal the Grantee's 2023-24 Comprehensive Primary Health Care agreement funding level plus a minimum five per cent increase applied.

The Grantee's funding in 2025-26 will be maintained at 2024-25 levels, with an added growth amount calculated using the IAHP Primary Health Care Funding Model.

In 2025-26, the department will use an average of the Grantee's reported OSR data from 2022-23, 2023-24 and 2024-25 to determine the level of additional growth that will be added to the funding in 2026-27 and 2027-28.

In 2026-27 and 2027-28, the Grantee's allocated funding will equal its 2025-26 allocated funding plus additional funds that would be determined as part of a funding review in 2025-26.

The department will advise the Grantee of the 2026-27 and 2027-28 funding levels by means of a Deed of Variation, no later than March 2026.

Performance Indicators

The Activity will be measured against the following Performance Indicator/s:

Performance Indicator Description	Measure
ACCHO provides First Nations people with access to culturally appropriate comprehensive primary health care services.	Number of clients registered in each service delivery location.
ACCHO delivers culturally appropriate, comprehensive primary health care that meets the health needs and priorities of the community.	Number of planned outputs listed in the Agreed Activity Work Plan achieved within the nominated timeframe.
ACCHO embeds Continuous Quality Improvement (CQI) and best practice clinical and organisational governance into CPHC service delivery.	Proportion of CQI goals in the AWP reported as achieved.

Location Information

The Activity will be delivered from the following site location/s:

	Location Type	Name	Address
1.	Direct Funded	Urapuntja Health Service Aboriginal Corporation	Sandover Highway UTOPIA NT 0872

Service Area Information

The Activity will service the following service area/s:

	Type	Service Area
1.	Primary Health Network (2015)	Northern Territory

C. Duration of the Grant

The Activity starts on 1 July 2024 and ends on 30 June 2028, which is the **Activity Completion Date**.

The Agreement ends on 30 November 2028 or when the Commonwealth accepts all of the reports provided by the Grantee and the Grantee has repaid any Grant amount as required under this Agreement, which is the **Agreement End Date**.

D. Payment of the Grant

The total amount of the Grant is \$8,236,317.59 excluding GST (if applicable).

A break down by Financial Year is below:

Financial Year	Amount (excl. GST if applicable)
2024-2025	\$3,970,160.01
2025-2026	\$4,266,157.58

The Grantee must ensure that the Grant is held in an account in the Grantee's name and which the Grantee controls, with an authorised deposit-taking institution authorised under the Banking Act 1959 (Cth) to carry on banking business in Australia.

The Grantee's nominated bank account into which the Grant is to be paid is:

BSB Number	035-303
Financial Institution	Alice Springs 19 Todd Mall WBC
Account Number	486175
Account Name	Urapuntja Health Service Aboriginal Corporation

The Grant will be paid in instalments by the Commonwealth in accordance with the agreed Milestones, and compliance by the Grantee with its obligations under this Agreement.

Milestone	Anticipated date	Amount (excl. GST)	GST (if applicable)	Total (incl. GST if applicable)
IAHP CPHC Q1 2024-25	5 July 2024	\$992,540.00	\$99,254.00	\$1,091,794.00
IAHP CPHC Q2 2024-25	5 October 2024	\$992,540.00	\$99,254.00	\$1,091,794.00
IAHP CPHC Q3 2024-25	5 January 2025	\$992,540.00	\$99,254.00	\$1,091,794.00
IAHP CPHC Q4 2024-25	5 April 2025	\$992,540.01	\$99,254.00	\$1,091,794.01
IAHP CPHC Q1 2025-26	5 July 2025	\$1,066,539.40	\$106,653.94	\$1,173,193.34
IAHP CPHC Q2 2025-26	5 October 2025	\$1,066,539.40	\$106,653.94	\$1,173,193.34
IAHP CPHC Q3 2025-26	5 January 2026	\$1,066,539.40	\$106,653.94	\$1,173,193.34
IAHP CPHC Q4 2025-26	5 April 2026	\$1,066,539.38	\$106,653.94	\$1,173,193.32
Total Amount		\$8,236,317.59	\$823,631.76	\$9,059,949.35

Invoicing

The Grantee agrees to allow the Commonwealth to issue it with a Recipient Created Tax Invoice (RCTI) for any taxable supplies it makes in relation to the Activity.

E. Reporting

The Grantee agrees to create the following reports in the form specified and to provide the reports to the Commonwealth representative in accordance with the following.

Milestone	Information to be included	Due Date
Activity Work Plan	Activity Work Plan and Budget as described in E.2 covering the Activity period 1 July 2024 to 30 June 2025.	15 August 2024
Other Report	Annual statement of compliance with Commonwealth Child Safe Framework, as described at E.4.	31 October 2024

Other Report	National Key Performance Indicators (nKPIs) data as described in item E.4, covering the period from 1 July 2024 to 31 December 2024.	15 February 2025
Activity Work Plan	Activity Work Plan and Budget as described in E.2 covering the Activity period 1 July 2025 to 30 June 2026.	31 July 2025
Other Report	National Key Performance Indicators (nKPIs) data covering the period 1 January 2025 to 30 June 2025, and OSR data covering the period from 1 July 2024 to 30 June 2025 as described in E.4.	15 August 2025
Performance Report	Report detailing achievements against the objectives in the Agreed Activity Work Plan during the reporting period 1 July 2024 to 30 June 2025 as described in E.1.	31 October 2025
Other Report	Annual statement of compliance with Commonwealth Child Safe Framework, as described at E.4.	31 October 2025
Financial Acquittal Report	Financial Declaration and an Audited Income and Expenditure Statement for the 2024-25 financial year as described in E.3.	31 October 2025
Other Report	National Key Performance Indicators (nKPIs) data as described in item E.4, covering the period from 1 July 2025 to 31 December 2025.	15 February 2026
Activity Work Plan	Activity Work Plan and Budget as described in E.2 covering the Activity period 1 July 2026 to 30 June 2027.	31 July 2026
Other Report	National Key Performance Indicators (nKPIs) data covering the period 1 January 2026 to 30 June 2026, and OSR data covering the period from 1 July 2025 to 30 June 2026 as described in E.4.	15 August 2026
Financial Acquittal Report	Financial Declaration and an Audited Income and Expenditure Statement for the 2025-26 financial year as described in E.3.	31 October 2026
Performance Report	Report detailing achievements against the objectives in the Agreed Activity Work Plan during the reporting period 1 July 2025 to 30 June 2026 as described in E.1.	31 October 2026
Other Report	Annual statement of compliance with Commonwealth Child Safe Framework, as described at E.4.	31 October 2026
Other Report	National Key Performance Indicators (nKPIs) data as described in item E.4, covering the period from 1 July 2026 to 31 December 2026.	15 February 2027
Activity Work Plan	Activity Work Plan and Budget as described in E.2 covering the Activity period 1 July 2027 to 30 June 2028.	31 July 2027

Other Report	National Key Performance Indicators (nKPIs) data covering the period 1 January 2027 to 30 June 2027, and OSR data covering the period from 1 July 2026 to 30 June 2027 as described in E.4.	15 August 2027
Financial Acquittal Report	Financial Declaration and an Audited Income and Expenditure Statement for the 2026-27 financial year as described in E.3.	31 October 2027
Performance Report	Report detailing achievements against the objectives in the Agreed Activity Work Plan during the reporting period 1 July 2026 to 30 June 2027 as described in E.1.	31 October 2027
Other Report	Annual statement of compliance with Commonwealth Child Safe Framework, as described at E.4.	31 October 2027
Other Report	National Key Performance Indicators (nKPIs) data as described in item E.4, covering the period from 1 July 2027 to 31 December 2027.	15 February 2028
Other Report	National Key Performance Indicators (nKPIs) data covering the period 1 January 2028 to 30 June 2028, and OSR data covering the period from 1 July 2027 to 30 June 2028 as described in E.4.	15 August 2028
Financial Acquittal Report	Financial Declaration and an Audited Income and Expenditure Statement for the 2027-28 financial year as described in E.3.	31 October 2028
Performance Report	Report detailing achievements against the objectives in the Agreed Activity Work Plan during the reporting period 1 July 2027 - 30 June 2028 as described in E.1.	31 October 2028

E.1 Performance Reports

The Grantee must provide Performance Reports to the department via the Health Data Portal (a web-based reporting system provided by the Commonwealth) at the time specified in Milestone Table at item E. Performance reports detail progress towards meeting the planned deliverables and objectives of the Agreed Activity Work Plan and must:

- Include evidence of your progress towards completion of the planned deliverables and objectives of the Agreed Activity Work Plan; and
- be submitted by the report due date.

E.2 Activity Work Plan

The Grantee must provide an annual Activity Work Plan via the Health Data Portal.

The Activity Work Plan must identify the activities that the Grantee will be deliver using the grant funding during the ensuing financial year. The Activity Work Plan must detail:

- a) How your organisation will fulfil the Activity Objectives specified in Item B;
- b) The budget for the Activity;
- c) Strategies to meet the obligations of the Activity;

- d) How nKPIs and NT AHKPIs will be incorporated into planned service delivery;
- e) Proposed performance indicators/targets and outcomes;
- f) Risks and/or barriers identified for the Activity, and how your organisation will minimise or overcome them.

An Activity Work Plan submitted by the grantee that is approved by the Commonwealth will, on approval, become the Agreed Activity Work Plan for this Project.

E.3 Financial Acquittal Reports

Financial Declaration and Audited Income and Expenditure Statement

The Grantee is required to submit to the Commonwealth, a Financial Declaration, on a template provided, and an Audited Income and Expenditure Statement every 12 months during the term of the Activity in accordance with the due dates specified at Item E.

The Financial Declaration and Audited Income and Expenditure Statement must be signed by the Grantee and certify that the Grantee has spent grant funding to perform the activity as set out in the Agreement. Where applicable, the Grantee must include in the certification the details of any unspent funds.

The Audited Income and Expenditure Statement must be prepared by

- a) a Registered Company Auditor under the Corporations Act 2001 (Cth.); or
- b) a member of CPA Australia; or
- c) a member of the Institute of Public Accountants in Australia; or
- d) a member of the Institute of Chartered Accountants in Australia.

The Audited Income and Expenditure Statement is to be accompanied by the audit opinion.

The Audited Income and Expenditure Statement must:

- be prepared in accordance with the applicable Australian Accounting Standards;
- be based on proper accounts and records for the grant recipient;
- verify that grant funding was spent to perform the activity as set out in the Agreement; and
- Include any other matters as specified in the Agreement

E.4 Other Reports

Online Services Report (OSR) and National Key Performance Indicators (nKPIs)

Grantees must submit an Online Services Report (OSR) and National Key Performance Indicators (nKPIs) via the Health Data Portal, a web-based reporting system provided by the Commonwealth, in accordance with the dates specified in Item E.

a) Online Services Report (OSR) – Grantees provide details of organisational data, including staff numbers, client numbers and episodes of care over the previous financial year. Completed reports are due 15 August each year.

b) National Key Performance Indicators (nKPIs) – Grantees report twice per year on their organisational achievement against a set of KPIs, developed by the Commonwealth in consultation with the Aboriginal Community Controlled Health Services (ACCHS) sector. Reports are due 15 February and 15 August each

year.

Each report must be complete and considered satisfactory by the Department before the Milestone will be met. There may be instances where the Australian Institute of Health and Welfare (AIHW) asks the Grantee to review and resubmit corrected OSR or nKPI data. In those instances, the AIHW will initiate Exception reporting by sending the OSR or nKPI data asset back to the Grantee for amendment via the Health Data Portal and the Grantee must resubmit the data asset via the Health Data Portal no later than two weeks after the original request was received.

The Grantee must:

- a) Install and maintain an appropriate Clinical Information System (CIS) to collect, store and retrieve reliable data for each of the nKPI indicators.
- b) Provide sufficient staff training to support the operation of the CIS.
- c) Collect data according to the specifications set out in the data portal.

The Commonwealth will utilise finalised nKPI data and OSR data at the service level for monitoring, mapping and planning purposes in accordance with the principles outlined in the Aboriginal and Torres Strait Islander Primary Health Care Data Framework.

A Grantee's OSR data will be used to inform future funding calculations under the IAHP Primary Health Care Funding Distribution Model methodology. Unless otherwise agreed with the Commonwealth, failure to submit the OSR data by the due date may affect the Grantee's eligibility for funding extensions under the rolling agreement.

Northern Territory Aboriginal Health Key Performance Indicators (NT AHKPI)

The Grantee will :

- a) Install and maintain an appropriate Clinical Information System (CIS) to collect, store and retrieve reliable data for each of the NT AHKPI indicators.
- b) Provide sufficient staff training to support the operation of the CIS.
- c) Collect data according to the specifications set out in the NT AHKPI Information System.

Statement of Compliance with Commonwealth Child Safe Framework

Grantees must provide the Commonwealth an annual statement of compliance report using the form issued by the Hub for this purpose, consistent with the requirements under Supplementary Terms and Conditions clause CB9.3 (f) National Principles for Child Safe Organisations and other actions for the safety of children must be submitted. A Statement of Compliance Report ensures compliance with relevant State, Territory and Commonwealth legislation, including Working with Children Checks, and with the National Principles for Child Safe Organisations. The report must reflect the Grantee has met the conditions as outlined in the Supplementary Terms CB9.2 and CB9.3 of this Agreement.

F. Party representatives and address for notices

Grantee's representative and address

Grantee's representative name	Dr Melissa Hinson
Position	Chief Executive Officer
Business hours telephone	08 8956 9875
E-mail	ceo@urapuntja.org.au

Commonwealth representative and email address

Business hours telephone	not applicable
E-mail	NTperformanceHealth@communitygrants.gov.au

The Parties' representatives will be responsible for liaison and the day-to-day management of the Grant, as well as accepting and issuing any written notices in relation to the Grant.

Disclaimer and explanatory notes

Disclaimer

By executing this agreement you agree that you have read and accept this disclaimer, including the explanatory notes on how to duly execute this agreement. You warrant that your identity has been verified, you have legal capacity and authority to enter into this agreement, and you are signing in accordance with all legal instruments that apply to you and/or the legal entity which you represent.

Explanatory notes

- If you are an **individual**, you must download, print and sign the agreement in wet-ink in the presence of a witness (the witness date must be the same as the signatory date).
- If you are a **partnership**, the signatory must be all partners, or one partner with the authority to sign on behalf of all partners receiving the grant. You should be prepared to provide evidence of this authorisation upon request.
- If you are a **proprietary company incorporated under the Corporations Act 2001 (Cth)**, the signatory must be the sole director and company secretary, as required under section 127 of the *Corporations Act 2001 (Cth)*. If required by your Constitution, please affix your **company seal** in the presence of the sole director and company secretary acting as a witness (the witness date must be the same as the date the company seal is affixed). For execution by company seal, you must download, print and sign the agreement in wet-ink.
- If you are a **company incorporated under the Corporations Act 2001 (Cth)**, the signatories must be two directors, or one director and one company secretary, as required under section 127 of the *Corporations Act 2001 (Cth)*. If required by your Constitution, please affix your **company seal** in the presence of two directors, or one director and one company secretary acting as a witness, or if your company has only one director – that director and a suitable witness, (the witness date must be the same as the date the company seal is affixed). For execution by company seal, you must download, print and sign the agreement in wet-ink.
- If you are a **company incorporated under the Corporations (Aboriginal and Torres Strait Islander) Act 2006 (Cth) (CATSI Act)**, the signatories must be two directors, one director and one company secretary, or if your company has only one director – that director, as required under section 99-5 of the CATSI Act. If required by your Constitution, please affix your **company seal** in the presence of two directors, or one director and one company secretary, or if your company has only one director – that director. For execution by company seal, you must download, print and sign the agreement in wet-ink.
- If you are an **individual trustee of a trust**, you must download, print and sign the agreement in wet-ink in the presence of a witness (the witness date must be the same as the signatory date). You must sign in your capacity as a trustee and not the trust. The trustee is the legal entity entering into the agreement. The words ‘as trustee for [name of trust]’ should be included in the signature block.
- If you are a **corporate trustee of a trust**, the signatory must be the sole director and company secretary in the proprietary company, or two directors in the company, or one director and one company secretary in the company, as required under section 127 of the *Corporations Act 2001 (Cth)*. If required by your Constitution, please affix your **company seal** in the presence of the sole director and company secretary in the proprietary company, or two directors in the company, or one director and one company secretary in the company, acting as witness (the witness date must be the same as the date the company seal is affixed). For execution by company seal, you must download, print and sign the agreement in wet-ink. The company must sign in its capacity as a trustee and not the trust. The trustee is the legal entity entering into the agreement. The words ‘as trustee for [name of trust]’ should be included in the signature block.

- If you are an **incorporated association**, you must refer to the legislation incorporating the association as it will specify how documents must be executed. This process may differ between each State and Territory. If an authorised person is executing a document on behalf of the incorporated association, you should be prepared to provide evidence of this authorisation upon request.
- If you are a **university**, the signatory can be an officer authorised by the legislation creating the university to enter into legally binding documents. A witness to the signature is required (the witness date must be the same as the signatory date).

Organisation ID:	2-M9NY76
Agreement ID:	4-JOVK9IV
Program Schedule ID:	4-JOVK9IX

Signatures

Executed as an Agreement

Signed for and on behalf of the Commonwealth of Australia by the relevant Delegate, represented by and acting through Department of Health and Aged Care, ABN 83 605 426 759 in the presence of:

(Name of Departmental Representative)

(Signature of Departmental Representative)

.../.../.....

(Position of Departmental Representative)

(Name of Witness in full)

(Signature of Witness)

.../.../.....

Signed for and on behalf of Urapuntja Health Service Aboriginal Corporation, ABN 45 449 518 275 in accordance with its rules, and who warrants they are authorised to sign this Agreement:

(Name and position held by Signatory)

(Signature)

.../.../.....

(Name and position held by second Signatory/Name of Witness)

(Signature of second Signatory/Witness)

.../.../.....